Printable Forms

Mental Health Group Therapy Curriculum for IOP and PHP · Volume 1

Appendix D: Forms 1 to 8, including the Rotation Passport, the session and handoff forms, the GIRP and DAP note forms, and the crisis and support contact card.

Appendix G: Program setup. Who to call, the staff handoff form, who can lead which sessions, and the monthly program check.

Book page numbers are kept at the bottom of each page so they match the printed book. Print on US Letter at actual size, or choose "Fit to printable area" on A4.

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Appendix D: Eight Reproducible Forms

Printable masters. Form 1 has five pages and Form 4 has two. Fill in your program's details and check them against your privacy, consent, records, and emergency procedures before first use. Blank lines are for writing. Clean copies are free to download from thoughtkindpress.com/v1 (see "Printable forms" at the front of the book).

Form 1: Rotation Passport - Process

Client-held | Passport page 1 of 5 | Attendance is not a test of mastery.

Program-approved identifier (may still be identifiable): _____ Admission date: _____ IOP / PHP

_____

Code	Topic	Date attended / revisit	Staff initials
PR-01	Arriving	_____	_____
PR-02	Carrying the Week	_____	_____
PR-03	The Mask and the Mirror	_____	_____
PR-04	Frustration and Stuckness	_____	_____
PR-05	Trust and Safety Here	_____	_____
PR-06	The Inner Critic	_____	_____
PR-07	Grief, Loss, and What Changed	_____	_____
PR-08	Hope and Doubt	_____	_____
PR-09	Connection and Isolation	_____	_____
PR-10	Resilience and Quiet Wins	_____	_____
PR-11	Giving and Receiving Feedback	_____	_____
PR-12	Anger and Boundaries	_____	_____
PR-13	Shame and Self-Forgiveness	_____	_____
PR-14	Identity Beyond the Diagnosis	_____	_____
PR-15	Navigating Outside Relationships	_____	_____
PR-16	Setbacks and Recovery Momentum	_____	_____
PR-17	Preparing for Transition / Stepping Down	_____	_____
PR-18	Endings, Goodbyes, and Continuity	_____	_____

An attendance date records attendance only. Note a revisit as such; do not infer understanding, clinical readiness, or completion of a full treatment model. Keep this document private. You may request staff help with recording or reading it.

Form 1: Rotation Passport - Skills

Client-held | Passport page 2 of 5 | Attendance is not a test of mastery.

Program-approved identifier (may still be identifiable): _____ Admission date: _____ IOP / PHP

_____

Code	Topic	Date attended / revisit	Staff initials
SK-MF-01	Observe and Describe	_____	_____
SK-MF-02	Grounding When Overwhelmed	_____	_____
SK-MF-03	Attention Training	_____	_____
SK-ER-01	Emotions, Urges, and Actions	_____	_____
SK-ER-02	Check the Facts and Opposite Action	_____	_____
SK-ER-03	Reducing Emotional Vulnerability	_____	_____
SK-DT-01	Crisis Survival Skills	_____	_____
SK-DT-02	Radical Acceptance and Turning the Mind	_____	_____
SK-DT-03	Self-Soothing with the Senses and a Distraction Plan	_____	_____
SK-CG-01	The Thought-Feeling-Behavior Connection and Common Thinking Traps	_____	_____
SK-CG-02	Examining the Evidence and Thought Reframing	_____	_____
SK-CG-03	Cognitive Defusion and Noticing Hooked Thoughts	_____	_____
SK-BA-01	Activity Scheduling and the Pleasure-Mastery Balance	_____	_____
SK-BA-02	Values-Based Action and Overcoming Avoidance	_____	_____
SK-BA-03	Graded Task Assignment and Habit Stacking	_____	_____
SK-IE-01	Objective Effectiveness and Assertive Communication	_____	_____
SK-IE-02	Relationship Effectiveness and Boundary Setting	_____	_____
SK-IE-03	Self-Respect Effectiveness and Saying No	_____	_____

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Form 1: Rotation Passport - Psychoeducation

Client-held | Passport page 3 of 5 | Attendance is not a test of mastery.

Program-approved identifier (may still be identifiable): _____ Admission date: _____ IOP / PHP

_____

Code	Topic	Date attended / revisit	Staff initials
PE-01	Understanding Depression	_____	_____
PE-02	Understanding Anxiety and the Fight-or-Flight Response	_____	_____
PE-03	Chronic Stress and Recovery Time	_____	_____
PE-04	The Panic Cycle and Interoceptive Conditioning	_____	_____
PE-05	Sleep and Mood	_____	_____
PE-06	Strong Emotions and Making Room to Pause	_____	_____
PE-07	The Window of Tolerance and Autonomic Regulation	_____	_____
PE-08	Understanding Trauma, Triggers, and the Nervous System	_____	_____
PE-09	Shame, Self-Criticism, and Self-Compassion	_____	_____
PE-10	Values, Meaning, and Personal Direction	_____	_____
PE-11	Understanding Anger and Emotional Triggers as Functional Signals	_____	_____
PE-12	Grief, Loss, and Non-Death Transitions	_____	_____
PE-13	Loneliness and Meaningful Connection	_____	_____
PE-14	Interpersonal Boundaries and Relationship Systems	_____	_____
PE-15	Executive Dysfunction, Task Paralysis, and Motivation	_____	_____
PE-16	Building Resilient Support Networks and Crisis Navigation	_____	_____
PE-17	Relapse Prevention: Lapses, Relapses, and Recovery Curves	_____	_____
PE-18	Stepping Down: Maintenance, Care Continuity, and Generalization	_____	_____

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Form 1: Rotation Passport - PHP Extension

Client-held | Passport page 4 of 5 | Attendance is not a test of mastery.

Program-approved identifier (may still be identifiable): _____ Admission date: _____ IOP / PHP

_____

Code	Topic	Date attended / revisit	Staff initials
PH-WS-01	Daily Rhythm Anchors	_____	_____
PH-WS-02	Eating Routines, Access, and Daily Energy	_____	_____
PH-WS-03	Building an Evening Wind-Down	_____	_____
PH-WS-04	Daily Demands and Energy Pacing	_____	_____
PH-WS-05	Medication Adherence and Prescriber Partnership	_____	_____
PH-WS-06	Rest, Withdrawal, and What I Need	_____	_____
PH-SP-01	Deconstructing the Crisis Trajectory: Micro-Triggers and Prodromal Signs	_____	_____
PH-SP-02	Internal Coping vs. Social Distraction	_____	_____
PH-SP-03	Environmental Safety and Means Restriction	_____	_____
PH-SP-04	The Anatomy of an Urge: Urge Surfing and Cognitive Decoupling	_____	_____
PH-SP-05	Post-Lapse Cognitive Processing: Reducing the Abstinence Violation Effect	_____	_____
PH-SP-06	Reviewing Safety-Plan Access, Contacts, and Gaps	_____	_____
PH-TS-01	Questions About Levels of Care and Stepping Down	_____	_____
PH-TS-02	Roles and Identity Beyond Treatment	_____	_____
PH-TS-03	My Routine and Check-In Plan	_____	_____
PH-TS-04	Outpatient Follow-Up and Handoff Questions	_____	_____
PH-TS-05	Mixed Feelings About Leaving	_____	_____
PH-TS-06	My Recovery Reference	_____	_____
PH-AP-01	Practicing a Request or Boundary	_____	_____
PH-AP-02	Noticing Mild Body Sensations and Grounding	_____	_____
PH-AP-03	Working With a Manageable Current Thought	_____	_____
PH-AP-04	Taking a First Step in the Room	_____	_____
PH-AP-05	Safety-Plan Access Practice	_____	_____
PH-AP-06	Adapting a Skill for Daily Life	_____	_____

An attendance date records attendance only. Note a revisit as such; do not infer understanding, clinical readiness, or completion of a full treatment model. Keep this document private. You may request staff help with recording or reading it.

Form 1: Passport - Special Plans and Personal Notes

Client-held | Passport page 5 of 5

Program-approved identifier:

_____

Code	Topic	Date / not applicable	Staff initials
SP-OR	Day-One Program Orientation and Expectation Setting	_____	_____
SP-SD	A Private Transition Review and Optional Group Goodbye	_____	_____
SP-CD	Support After a Difficult Group Event	_____	_____
SP-AV	Low Attendance and Program Disruption	_____	_____

PHP track coverage - count distinct codes actually attended, with staff assistance if wanted:

Track	Distinct codes attended (0-6)	Questions / personal notes
Wellness and daily structure	_____	
Safety and relapse planning	_____	
Transition and step-down	_____	
Applied practice	_____	

Notes to myself:

_____

_____

_____

_____

_____

_____

_____

Bring your Passport when it is useful. At transition, review gaps privately with staff. Take-home information does not replace a session, individualized care, or a reviewed clinical safety plan.

Form 2: Attendance and Rotation Slot Tracker

Staff-held | One program day | Store in an approved restricted location.

Program / date / core day index / extension day index / staff:

_____

_____

Slot	Planned code	Actual code / format	Actual start-stop
G1			
G2			
G3			
G4			
G5			

Client ID	G1	G2	G3	G4	G5	New / transition
_____	____	____	____	____	____	_____
_____	____	____	____	____	____	_____
_____	____	____	____	____	____	_____
_____	____	____	____	____	____	_____
_____	____	____	____	____	____	_____
_____	____	____	____	____	____	_____
_____	____	____	____	____	____	_____
_____	____	____	____	____	____	_____
_____	____	____	____	____	____	_____
_____	____	____	____	____	____	_____

Use program-approved attendance codes and distinguish absent, present, partial, declined, and not offered. Actual service times and individual notes are maintained in the required clinical records.

Closure / substitution / reason and next operational slot:

_____

_____

_____

Completed by / reviewed by / date:

_____

Form 3: Group Agreement and Confidentiality Discussion

Review with staff at orientation | Offer a copy | Supplement with the program's required informed consent.

Client identifier / staff name and role / date:

_____

Please ask questions or request an accessible explanation. This page acknowledges a discussion; it does not waive care, privacy rights, or the right to raise concerns.

Privacy and its limits

[ ] I am asked not to repeat identifiable information about others, record, photograph, or share screenshots. Staff cannot guarantee that other participants will keep information private. I can ask what participation options fit my privacy needs.

[ ] Staff explained relevant treatment-team sharing, record access, and applicable reporting, emergency, or protective exceptions. The program's jurisdiction-specific consent process governs those details.

Program-specific limits explained / questions answered / language or interpreter support:

_____

_____

_____

Participation, respect, and access

[ ] I can choose what to disclose and may request listening, writing, demonstration, an adapted activity, or a supported pause. Attendance and treatment expectations, and any limits on these choices, have been explained separately. I will not threaten, harass, or pressure another person to disclose.

[ ] We ask permission before advice or feedback. Peers do not provide risk assessment, medication advice, or crisis care. Staff retain responsibility for clinical decisions.

[ ] Staff explained how to reach help during group, between sessions, and in an emergency. My current contact card is available in an accessible form.

Local norms, access needs, camera/chat/recording arrangements if relevant:

_____

_____

_____

Teach-back: how I can request help or a private concern:

_____

_____

Client acknowledgment / date / staff acknowledgment / date (or document verbal review per policy):

_____

_____

Form 4: Session Delivery and Secure Handoff

Staff-held | Group operations and pending clinical actions | Keep personal details in approved clinical channels.

Date / facilitator / co-facilitator / planned and actual code / actual format and time:

_____

_____

What was actually delivered; adaptations; attendance count; displaced code or closure:

_____

_____

_____

Group dynamic or access issue relevant to the next facilitator (avoid unnecessary identifiers):

_____

_____

_____

Closed-loop clinical handoff - use minimum necessary information under applicable policy

Client ID / secure record pointer	Concern / action already taken	Receiving clinician / due time	Acknowledged / contingency

Immediate unresolved issue, next action, responsible person, acknowledgment time:

_____

_____

_____

An urgent concern requires direct clinical communication now, not a form left for the next shift. Record clinical assessment and actions in the required individual record; do not place another member's information in a patient's note.

Outgoing staff / time; incoming staff / time; acknowledgment or alternate recipient:

_____

_____

Form 4: Individual Encounter Note - GIRP or DAP

One person and one actual encounter | Alternative formats; choose the format required locally.

Client ID / date / actual service, code, location, and start-stop time / clinician:

_____

_____

GIRP fields

G - Individual treatment goal and relevance of this encounter:

_____

_____

I - Intervention actually delivered, dose/time, accommodations, and format:

_____

_____

_____

R - Observed behavior / performance / declined or not assessed; then separately attributed member report:

_____

_____

_____

_____

P - Agreed next step or no step agreed; assistance, referral, owner, due time, acknowledgment:

_____

_____

_____

DAP alternative

D - Actual intervention, attendance/time, observed behavior, and attributed report:

_____

_____

_____

A - Interpretation linked to an individual goal, with the supporting observation; state limits:

_____

_____

P - Individual next step and any clinical action, owner, due time, handoff:

_____

_____

Do not infer improvement, safety, engagement, understanding, or readiness from attendance or a worksheet. Record urgent actions as they occur. This is not a completed note, validated assessment, or billing assurance.

Form 5: Coverage Review and Continuity Assignment

Staff and member, privately | At transition or a planned review | Coverage is separate from mastery.

Client ID / date / reviewed with / receiving provider if applicable:

_____

_____

Codes actually offered during stay: _____ Distinct codes attended: _____ Revisits: _____

_____

Code / gap	Summary or practice material offered	Date / accessible format	Clinical review or follow-up owner

Skills summaries in the companion workbook may support a discussion, but do not replace missed treatment. Select only material that fits the member's needs and the receiving clinician's plan. Do not issue a self-guided exposure task or an incomplete safety plan as a catch-up assignment.

Reason for gap, if useful for program review (not a failure label):

_____

_____

Appointment accepted / date / access barriers / responsible staff / due time:

_____

_____

_____

Receiving-service contact and handoff acknowledgment, under the applicable consent process:

_____

_____

Form 6: Crisis and Support Contact Card

Client-held | Complete local fields with staff, verify, and cut or copy the card | A contact card is not a safety plan.

Staff check before issue: verify location served, hours, eligibility, emergency route, accessible format, and backup. Do not hand out blank program contact fields as if they were verified resources.

CRISIS AND SUPPORT CARD - FRONT	MY SUPPORT CONTACTS - BACK
United States: call or text 988 for the Suicide & Crisis Lifeline. For immediate danger or a medical emergency, contact local emergency services (911 in the U.S.) or follow the emergency route confirmed by your care team. Program: _____ Daytime: _____ After-hours / hours served: _____ Local emergency route: _____	My clinician: _____ Phone / hours: _____ Prescriber: _____ Phone / hours: _____ Support person, if agreed: _____ Phone / availability: _____ Backup service / hours: _____ Current reviewed plan location: _____ Date checked with staff: _____

Outside the United States, staff must substitute current resources for the participant's actual location. A phone being unavailable, a contact not answering, or a coping step not helping is a reason to use the backup or urgent help route, not to wait through a required sequence.

Verified by / date / participant location served / accessible version and backup:

_____

_____

_____

Form 7: Safety Escalation and Follow-Up

Staff-held | Supplement the required risk/incident record | Immediate communication when urgent.

Client ID / date / session code / facilitator / actual time concern arose:

_____

_____

Observed facts and separately attributed statement (no unsupported interpretation):

_____

_____

_____

_____

Immediate action / times / staff assigned / medical or emergency response if used:

_____

_____

_____

_____

Individual assessment completed by / secure record location / current disposition:

_____

_____

_____

Receiving clinician or supervisor / notification method and time / acknowledgment:

_____

_____

Unresolved action / owner / due time / contingency if the recipient cannot be reached:

_____

_____

_____

Participant follow-up and next contact; group support needs without other-member details:

_____

_____

_____

Facilitator signature/time / supervisor review/time:

_____

_____

Do not treat a checkbox, a promise, a grounding response, or a returned worksheet as evidence of low risk. The responsible clinician and program procedures determine assessment, monitoring, escalation, and follow-up.

Form 8: Your Experience of the Program

Optional | No name requested | Participation does not affect care.

Do not include another person's identifying information. Staff should explain how responses are collected and any limits on anonymity, especially in a small program. This survey is an editorial feedback aid, not a validated outcome measure.

Please circle one number, N/A, or skip any item. 1 = not helpful; 5 = very helpful. These are experience questions, not symptom scores.

Experience	1	2	3	4	5	N/A
Understanding how to join the active rotation						
Practical usefulness of the groups						
Being able to choose how to participate						
Facilitator response to my needs						
Usefulness of any workbook or handouts						
Preparation for the next stage of care						

What was useful for you?

_____

_____

_____

_____

What would you change or want handled differently?

_____

_____

_____

_____

Anything the program did not address, or an access need it missed?

_____

_____

_____

Optional context: IOP / PHP / prefer not to say. Please omit identifying details. For an urgent concern, contact program staff or the verified help route directly; this form is not monitored for emergency response.

Appendix G: Program Setup

Four pages to work through with your clinical lead before the first group, and to revisit when staff or phone numbers change. Copy them for staff binders. They're starting points for your own procedures, not a substitute for them.

G.1 Who to call

Use roles and phone numbers, not client names.

Situation	Who I call, and how	Backup if no answer
Help during group (any concern)		
Possible suicide or self-harm risk		
Medical emergency, overdose, seizure, possible withdrawal		
Threat, violence, or an unsafe room		
Safeguarding or mandated-reporting question		
Medication question or possible side effect		
Interpreter or language support		
Video group: member disconnects or may be unsafe		
After hours (for members)		

Group size and staffing

Our maximum group size: _____ When two facilitators are needed: _____

If the planned backup is missing, change the plan before group starts: find another person, shorten the session, or reschedule. An emergency that happens anyway still gets the emergency response.

Before each group

□ I know who comes to stay with the group if I have to step out. If nobody can, I don't step out, and the plan is: _____

□ I have a made-up example ready for today's skill, and the plain-paper prompts if workbooks aren't available.

□ For video: I have each member's current location and a callback number, and I know the emergency service for that location.

□ I know where notes go and what happens to completed worksheets.

□ We have practiced the call-for-help route at least once this quarter.

Reviewed by: _____ Date: _____ Next review: _____

G.2 Staff handoff

Use this, or your clinical system, whenever a concern passes from one staff member to another. A client's Passport or worksheet is never the handoff. The handoff is done when the receiver confirms what they are taking on.

From (name, role): _____ To (name, role): _____ Date and time: _____

What I saw and heard (exact words in quotation marks)

_____

_____

_____

What I did, and how the person responded

_____

_____

_____

What I'm worried about, and what I don't know yet

_____

_____

Next step, who does it, by when, and what happens if it doesn't happen

_____

_____

_____

Receiver: I understand and I'm taking on

_____

_____

Write only what was observed or reported. Don't copy forward claims of engagement, improvement, or consent that weren't there. Leave other members' stories and names out unless something specific is needed for safety.

G.3 Who can lead which sessions

Every session needs a qualified facilitator, a decision that the session suits the people in the room, and a known way to get help. The last column adds what matters most for each session. Your program decides who may lead each one and when a co-facilitator or supervisor must be present; write that decision on your schedule. The Vol. 2 column gives the matching workbook page.

Code	Session	Vol. 2	What the lead needs
PR-01	Arriving	48	Process-group facilitation
PR-02	Carrying the Week	49	Process-group facilitation
PR-03	The Mask and the Mirror	50	Process-group facilitation
PR-04	Frustration and Stuckness	51	Process-group facilitation
PR-05	Trust and Safety Here	52	Process-group facilitation
PR-06	The Inner Critic	53	Process-group facilitation
PR-07	Grief, Loss, and What Changed	54	Process-group facilitation
PR-08	Hope and Doubt	55	Process-group facilitation
PR-09	Connection and Isolation	56	Process-group facilitation
PR-10	Resilience and Quiet Wins	57	Process-group facilitation
PR-11	Giving and Receiving Feedback	58	Feedback only with permission; stop personal attacks
PR-12	Anger and Boundaries	59	Anger and conflict; no forced confrontation
PR-13	Shame and Self-Forgiveness	60	Process-group facilitation
PR-14	Identity Beyond the Diagnosis	61	Process-group facilitation
PR-15	Navigating Outside Relationships	62	Process-group facilitation
PR-16	Setbacks and Recovery Momentum	63	Process-group facilitation
PR-17	Preparing for Transition / Stepping Down	64	Process-group facilitation
PR-18	Endings, Goodbyes, and Continuity	65	Process-group facilitation
SK-MF-01	Observe and Describe	10	Skills or psychoeducation facilitation
SK-MF-02	Grounding When Overwhelmed	11	Skills or psychoeducation facilitation
SK-MF-03	Attention Training	12	Skills or psychoeducation facilitation
SK-ER-01	Emotions, Urges, and Actions	13	Skills or psychoeducation facilitation
SK-ER-02	Check the Facts and Opposite Action	14	Skills or psychoeducation facilitation
SK-ER-03	Reducing Emotional Vulnerability	15	Skills or psychoeducation facilitation
SK-DT-01	Crisis Survival Skills	16	Skills or psychoeducation facilitation
SK-DT-02	Radical Acceptance and Turning the Mind	17	Skills or psychoeducation facilitation
SK-DT-03	Self-Soothing with the Senses and a Distraction Plan	18	Skills or psychoeducation facilitation
SK-CG-01	The Thought-Feeling-Behavior Connection and Common Thinking Traps	19	Cognitive skills; don't debate unusual beliefs in group

Code	Session	Vol. 2	What the lead needs
SK-CG-02	Examining the Evidence and Thought Reframing	20	Cognitive skills; don't debate unusual beliefs in group
SK-CG-03	Cognitive Defusion and Noticing Hooked Thoughts	21	Cognitive skills; don't debate unusual beliefs in group
SK-BA-01	Activity Scheduling and the Pleasure-Mastery Balance	22	Activation work; watch for possible mania or medical issues
SK-BA-02	Values-Based Action and Overcoming Avoidance	23	Activation work; watch for possible mania or medical issues
SK-BA-03	Graded Task Assignment and Habit Stacking	24	Activation work; watch for possible mania or medical issues
SK-IE-01	Objective Effectiveness and Assertive Communication	25	Skills or psychoeducation facilitation
SK-IE-02	Relationship Effectiveness and Boundary Setting	26	Skills or psychoeducation facilitation
SK-IE-03	Self-Respect Effectiveness and Saying No	27	Skills or psychoeducation facilitation
PE-01	Understanding Depression	29	Skills or psychoeducation facilitation
PE-02	Understanding Anxiety and the Fight-or-Flight Response	30	Skills or psychoeducation facilitation
PE-03	Chronic Stress and Recovery Time	31	Skills or psychoeducation facilitation
PE-04	The Panic Cycle and Interoceptive Conditioning	32	No symptom induction; new physical symptoms go to medical review
PE-05	Sleep and Mood	33	Skills or psychoeducation facilitation
PE-06	Strong Emotions and Making Room to Pause	34	Skills or psychoeducation facilitation
PE-07	The Window of Tolerance and Autonomic Regulation	35	Skills or psychoeducation facilitation
PE-08	Understanding Trauma, Triggers, and the Nervous System	36	Trauma-informed lead, with co-lead or supervision; no trauma stories in group
PE-09	Shame, Self-Criticism, and Self-Compassion	37	Skills or psychoeducation facilitation
PE-10	Values, Meaning, and Personal Direction	38	Skills or psychoeducation facilitation
PE-11	Understanding Anger and Emotional Triggers as Functional Signals	39	Skills or psychoeducation facilitation
PE-12	Grief, Loss, and Non-Death Transitions	40	Skills or psychoeducation facilitation
PE-13	Loneliness and Meaningful Connection	41	Skills or psychoeducation facilitation
PE-14	Interpersonal Boundaries and Relationship Systems	42	Skills or psychoeducation facilitation
PE-15	Executive Dysfunction, Task Paralysis, and Motivation	43	Skills or psychoeducation facilitation
PE-16	Building Resilient Support Networks and Crisis Navigation	44	Skills or psychoeducation facilitation
PE-17	Relapse Prevention: Lapses, Relapses, and Recovery Curves	45	Skills or psychoeducation facilitation
PE-18	Stepping Down: Maintenance, Care Continuity, and Generalization	46	Skills or psychoeducation facilitation
PH-WS-01	Daily Rhythm Anchors	67	Skills or psychoeducation facilitation
PH-WS-02	Eating Routines, Access, and Daily Energy	68	No weighing, calorie targets, or food challenges

Code	Session	Vol. 2	What the lead needs
PH-WS-03	Building an Evening Wind-Down	69	Skills or psychoeducation facilitation
PH-WS-04	Daily Demands and Energy Pacing	70	Skills or psychoeducation facilitation
PH-WS-05	Medication Adherence and Prescriber Partnership	71	No medication advice; questions go to the prescriber
PH-WS-06	Rest, Withdrawal, and What I Need	72	Skills or psychoeducation facilitation
PH-SP-01	Deconstructing the Crisis Trajectory: Micro-Triggers and Prodromal Signs	73	Safety-plan support; the clinician owns risk decisions
PH-SP-02	Internal Coping vs. Social Distraction	74	Safety-plan support; the clinician owns risk decisions
PH-SP-03	Environmental Safety and Means Restriction	75	Clinician owns means-safety planning; no access details in group
PH-SP-04	The Anatomy of an Urge: Urge Surfing and Cognitive Decoupling	76	Safety-plan support; the clinician owns risk decisions
PH-SP-05	Post-Lapse Cognitive Processing: Reducing the Abstinence Violation Effect	77	Safety-plan support; the clinician owns risk decisions
PH-SP-06	Reviewing Safety-Plan Access, Contacts, and Gaps	78	Safety-plan support; the clinician owns risk decisions
PH-TS-01	Questions About Levels of Care and Stepping Down	79	Transition planning; the team owns level-of-care decisions
PH-TS-02	Roles and Identity Beyond Treatment	80	Transition planning; the team owns level-of-care decisions
PH-TS-03	My Routine and Check-In Plan	81	Transition planning; the team owns level-of-care decisions
PH-TS-04	Outpatient Follow-Up and Handoff Questions	82	Transition planning; the team owns level-of-care decisions
PH-TS-05	Mixed Feelings About Leaving	83	Transition planning; the team owns level-of-care decisions
PH-TS-06	My Recovery Reference	84	Transition planning; the team owns level-of-care decisions
PH-AP-01	Practicing a Request or Boundary	85	Feedback only with permission; low-risk situations
PH-AP-02	Noticing Mild Body Sensations and Grounding	86	No symptom induction or breath holding; external grounding offered
PH-AP-03	Working With a Manageable Current Thought	87	Cognitive skills; don't debate unusual beliefs in group
PH-AP-04	Taking a First Step in the Room	88	Member chooses the task; stop or adapt for distress
PH-AP-05	Safety-Plan Access Practice	89	Safety-plan support; the clinician owns risk decisions
PH-AP-06	Adapting a Skill for Daily Life	90	Skills or psychoeducation facilitation
SP-OR	Day-One Program Orientation and Expectation Setting	-	Role named by program leadership

Code	Session	Vol. 2	What the lead needs
SP-SD	A Private Transition Review and Optional Group Goodbye	-	Role named by program leadership
SP-CD	Support After a Difficult Group Event	-	Only after immediate needs are met; nobody retells the event
SP-AV	Low Attendance and Program Disruption	-	Role named by program leadership

G.4 Monthly program check

A short review for the program lead and facilitators. It's for improving the program, not for scoring staff, and it isn't an outcome measure. Keep it free of client names.

Month: _____ Reviewed by: _____

Question	What we found	What we'll change, who, by when
Did each group get a Bridge, a real choice about participation, one complete practice, and a proper close?		
Which sessions were shortened, swapped, or cancelled, and why?		
Any incidents or near misses? Were the handoffs closed?		
Which access needs did we miss (language, reading, hearing, sensory, mobility)?		
What did members say helped, or didn't (Form 8)?		
Are our phone numbers and backup routes still right?		

Attendance tells you who was in the room. It doesn't tell you who improved, and a completed checklist isn't evidence that the program works. Review patterns with supervision rather than judging staff on one hard day.